BFM Patient Evaluation Report — March 2026

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Executive Summary

Patient	Age/Sex	Urgency	Deal Breakers	Defining Issues
DH Denis Hiestand	76M	**5/5 CRITICAL**	7	Spike protein 11,050 U/mL · CKD 3a (eGFR 56) · Heart 2% · CRP 15.89 · pH 5.08 · VCS 2/32
GD Geraldine DeMartino	72F	**5/5 MAXIMUM**	7	All organs 1% · Bigeminy/Trigeminy · pH 4.89 · Physiological resources 4/600 · HPA exhaustion
GG Gregory Geiger	69M	**5/5 CRITICAL**	7	Paradoxical deep breathing (Vagus Trauma) · Proteinuria 3.0 + Kidneys 1% · VCS 8/32
JW Jennifer Weber	52F	**2.5/5**	5	Mildest presentation · Only VCS PASS (25/32) · Yellow-zone organs · Lower-left quadrant ANS depletion
TH Tim Haskins	53M	**5/5 MAXIMUM**	7	HbA1c 9.6 (uncontrolled T2D) · CKD 3a (eGFR 50) · AST 57 (liver deal breaker) · Dual "BE MORE ATTENTIVE" · Heart 1%

> ***Cyto Lower Gate:*** *All four critical patients (DH, GD, GG, TH) need Cyto Lower but* ***cannot yet run it.*** *VCS scores of 2/32, 3/32, 8/32, and 12/32 all indicate MSH is almost certainly <8. MSH must be lab-confirmed >8 before scheduling. Begin Pit A Support + Pars Intermedia + Hypothala now for all four.*

PATIENT DH — Denis Hiestand {#patient-dh}

Male | Age 76 | DOB: 06/08/1949 Test Dates: DePulse 11/11/2025 · HRV/Ortho/Valsalva 11/11/2025 · UA 11/11/2025 · LabCorp 11/19/2025

Urgency Rating: 5 / 5 — CRITICAL

Denis presents with catastrophic multi-system failure. Every organ on DePulse is in the red zone, with Heart at 2% (life-threatening threshold). System Energy of 11 places him in energetic debt. PSNS is -3.5 with absent vascular compensation — the autonomic nervous system is fundamentally dysregulated. pH of 5.08 indicates severe metabolic acidosis incompatible with normal cellular function. VCS of 2/32 confirms massive biotoxin burden or neurological impairment. hs-CRP of 15.89 signals extreme systemic inflammation (nearly 16× the high-risk threshold). SARS-CoV-2 spike protein antibody of 11,050 U/mL indicates persistent spike protein burden driving ongoing damage. eGFR of 56 confirms CKD Stage 3a, limiting detox capacity. LDL of 214 with HDL of 38 and elevated homocysteine create compounding cardiovascular risk in a man whose heart is already at 2% on DePulse. Delta brainwaves at 80% in a waking state indicate the brain is essentially in a sleep/survival pattern. This is a patient in systemic crisis requiring immediate, carefully sequenced intervention.

Diagnostic Data Summary

DePulse+ (Organ Bioelectric Scores)

Organ	Score	Status
Heart	2%	🔴 CRITICAL
Blood Vessels	6%	🔴 Critical
Kidneys	9%	🔴 Critical
Spleen	17%	🔴 Red
Lungs	14%	🔴 Red
Colon	16%	🔴 Red
Stomach	20%	🔴 Red
Liver	14%	🔴 Red
Gallbladder	21%	🔴 Red
Pancreas	24%	🔴 Red
Small Intestine	33%	🟡 Yellow

HRV / NervXpress

- **System Energy:** 11 (Energetic Debt)
- **Stress Response:** 7 (Worst possible)
- **Brainwaves:** Delta 80% · Theta 12% · Alpha 4% · Beta 2% · Gamma 1%
- **Quadrant:** UPPER-LEFT — high SNS, deeply negative PNS
- **Clinical note:** Delta >70% in waking state = low direct current assumed. Theta (12%) > Alpha (4%) = deal breaker.

Orthostatic Test

- Physical Fitness Level: **11/7** (near worst)
- Chronotropic reaction: 0.85 (SHARPLY DECREASES on standing)

- Vascular compensation: **ABSENT**
- PSNS activity: **-3.5** · SNS activity: +1.5
- POV supine=1, upright=1, Index of discrepancy=-2
- Machine conclusion: *"CHRONOTROPIC REACTION DECREASES SHARPLY IN RESPONSE TO ORTHOTEST WHILE VASCULAR COMPENSATION IS ABSENT"*

Valsalva Maneuver

- Valsalva Ratio: **1.24** (LOW — below 1.3 threshold)
- E/I Ratio: 1.01 (essentially flat — no respiratory sinus arrhythmia)
- PNS: sharply decreased; SNS: slightly elevated
- Deep breathing: **NO improvement in PNS** (remains negative)
- Assessment: Severe vagal tone impairment — likely spike protein neuropathy and/or biotoxin-mediated nerve injury

Urinalysis + Visual Contrast Scale

- **pH: 5.08** (CRITICAL — normal 7.0-7.5; ~100× more acidic than normal)
- Specific Gravity: 1.025 · Ketones: 4.0 Moderate · Protein: 1.0
- Magnesium: 15 · Ascorbate: 5.0 · **Uric Acid: 700** (elevated)
- **VCS: 2/32** — CRITICAL FAIL (passing = 24+/32)

LabCorp Blood Panel (11/19/2025)

Marker	Value	Status
Creatinine	1.32	🔴 HIGH (normal <1.17)
eGFR	56	🔴 LOW — CKD Stage 3a
Total Cholesterol	286	🔴 HIGH
Triglycerides	175	🔴 HIGH
HDL	38	🔴 LOW (normal >40)
LDL	214	🔴 HIGH
hs-CRP (Cardiac)	15.89	🔴 VERY HIGH (high risk >3.0)
Homocysteine	18.4	🔴 HIGH (normal 0-14.5)
Reverse T3	31.4	🔴 HIGH (normal 9.2-24.1)
Ceruloplasmin	31.3	🟡 Borderline HIGH
Hematocrit	51.3	🟡 HIGH (borderline polycythemia)
SARS-CoV-2 Spike Ab	**11,050 U/mL**	🔴 VERY HIGH POSITIVE
Vitamin D (25-OH)	41.9	🟡 Suboptimal (target 60-80)
Ferritin	389	🟡 Elevated (normal male 24-336)

Deal Breakers Triggered — All 7

Deal Breaker #1: SNS SWITCHED

Evidence: PSNS -3.5, SNS +1.5. Both stress and recovery dots in upper-left quadrant. Valsalva 1.24 (below 1.3). Deep breathing produces NO PNS improvement. The autonomic system is locked in sympathetic overdrive with parasympathetic collapse. At -3.5 PNS, this is not merely elevated SNS — it is switched. The system cannot self-correct.

- **Frequency:** SNS Balance 35@709
- **Priority:** HIGHEST — must precede all other protocols

Deal Breaker #2: ALPHA/THETA IMBALANCE + LOW DIRECT CURRENT

Evidence: Delta 80% (far exceeds 70% threshold), Theta 12% > Alpha 4%. Delta at 80% waking indicates extremely low direct current. Theta exceeding Alpha is an absolute deal breaker. The brain is operating in a survival/shutdown pattern.

- **Frequency:** Alpha Theta
- **Supplements:** Cell Synergy (mandatory — Delta >70%), D-Ribose

Deal Breaker #3: PNS NEGATIVE ZONE

Evidence: PSNS -3.5, Valsalva 1.24, zero PNS recovery with deep breathing. System Energy 11 (energetic debt). Stress Response 7 (worst).

- **Frequency:** PNS Support

Deal Breaker #4: VAGUS NERVE DYSFUNCTION

Evidence: Valsalva 1.24, E/I 1.01 (essentially flat — no respiratory sinus arrhythmia), PNS negative and non-responsive. Absent vascular compensation. Consistent with spike protein neuropathy and/or biotoxin nerve injury.

- **Frequencies:** Vagus Support → assess response → escalate to Vagus Trauma if no movement
- **Supplement:** Innovita Vagus Nerve form code

Deal Breaker #5: LOW HEART ON D-PULSE (2%)

Evidence: Heart at 2% — well below the 5% critical threshold. Blood Vessels at 6%. Combined with hs-CRP 15.89, LDL 214, homocysteine 18.4, and absent vascular compensation: acute cardiovascular danger.

- **Frequency:** Heart Health
- **Supplements:** Serculate, CoQ10

Deal Breaker #6: LOW pH (5.08)

Evidence: pH 5.08 — approximately 100× more acidic than normal. pH below 6.2 = mandatory Tri-Salts. At 5.08, cellular enzyme function, oxygen delivery, and mineral absorption are severely compromised. The Krebs cycle cannot run at this pH.

- **Supplements:** Cell Synergy + Tri-Salts (mandatory — non-negotiable)
- **Frequency:** Terrain (given severity, do not wait for supplements to move pH)

Deal Breaker #7: VCS FAILED (2/32)

Evidence: VCS 2/32 — catastrophic failure. Passing threshold is 24/32. Combined with spike protein 11,050, this likely represents spike-protein-driven biotoxic illness. The 2/32 virtually guarantees critically low MSH, which gates Cyto Lower.

- **Frequency:** Biotoxin
- **Supplements:** Pectasol-C (immediate); **Apex — DELAYED** until VCS improves (eGFR 56 limits detox throughput — redistribution risk)
- **CRITICAL NOTE:** MSH almost certainly <8 at VCS 2/32 → Cyto Lower is GATED until MSH confirmed >8

Frequency Protocols — Complete List

Phase 1: Deal Breakers (Weeks 1–4)

Priority	Frequency	Duration/Setting	Clinical Trigger	Sequence Note
1	**SNS Balance**	35 min @ 709 Hz	SNS switched, PSNS -3.5	Run first every session
2	**Alpha Theta**	Standard	Delta 80%, Theta > Alpha	After SNS Balance
3	**PNS Support**	Standard	PSNS -3.5 negative	Pairs with SNS Balance
4	**Vagus Support**	Standard	Valsalva 1.24, E/I 1.01	→ Vagus Trauma if no movement
5	**Heart Health**	Standard	Heart 2%	Critical organ
6	**Terrain**	Standard	pH 5.08, all organs red	Foundational terrain
7	**Biotoxin**	Standard	VCS 2/32	Biotoxin initiation

Phase 2: Organ/System Support (Weeks 2–6)

Frequency	Clinical Trigger
Kidney Repair / Kidney Vitality	eGFR 56 (CKD 3a), Kidneys 9%
Lung Support	Lungs 14%
Concussion	POTS pattern (no vascular compensation, ChMR decreases)
Vein Vitality	Absent vascular compensation, hematocrit 51.3
Small Intestine	SI 33%, HDL 38 (gut-liver axis)
Liver Tox	Liver 14%, HDL 38, high cholesterol
Pineal Support	Energy 11, Delta 80% (Five Levers — melatonin)
Leptin Resist	All organs red, Delta 80% (Five Levers — leptin)

Phase 3: Lab-Driven Protocols (Weeks 3–8)

Frequency	Clinical Trigger
Mito Leak	hs-CRP 15.89 (extreme systemic inflammation)

Ferritin Lower	Ferritin 389 (elevated)
Virus Recovery	SARS-CoV-2 Spike Ab 11,050 U/mL
Aldehyde Detox	Uric Acid 700
Pit A Support + Pars Intermedia	VCS 2/32 → MSH restoration (gates Cyto Lower)

Gated: Cyto Lower

Cannot run until MSH confirmed >8. Begin Pit A Support + Pars Intermedia + Hypothala immediately. MSH must be drawn and confirmed. Expected timeline: 6–12 weeks minimum.

Supplementation Stack

Immediate — Day 1 (Non-Negotiable)

Supplement	Dose	Indication	Timing
Cell Synergy	**2–3 scoops**	Delta 80% (low DC), pH 5.08, all organs red	Mix in water, any time
Tri-Salts	Per label, 2–3×/day	pH 5.08 — mandatory at <6.2	Between meals
Augmented NAC	**3 pills/day** for 5–6 months	Spike protein 11,050 U/mL	Source: zerospike.org. Non-negotiable duration.
Nattokinase	Per label	Spike protein / clot clearing	Caution if on anticoagulants
Serculate	Per label	Heart 2%, absent vascular compensation, POTS pattern	Daily
CoQ10 (ubiquinol)	High dose → target RBC CoQ10 >1.65	Heart 2%, hs-CRP 15.89	Ubiquinol preferred at age 76
Pectasol-C	Per label	VCS 2/32, cholesterol 286, LDL 214, kidney support	Daily
Vitamin D3	**10,000 IU** × 4–6 weeks	Vitamin D 41.9 (target 60–80)	With food
Vitamin K2 (MK-7)	**200 mcg per 1,000 IU D3**	Mandatory with D3 — prevents calcium misplacement	At 10,000 IU D = 2,000 mcg K2 daily

Add Within First 2 Weeks

Supplement	Dose	Indication	Notes
IP6 Gold	Per label	Ferritin 389	⚠️ **EMPTY STOMACH ONLY — 1 hr before or 2 hr after food**

Homocysteine Factor	Per label	Homocysteine 18.4	B6, B12, methylfolate
Integra Cell	Per label	hs-CRP 15.89	Anti-inflammatory
Kidney Clear	Per label	eGFR 56, Kidneys 9%	Pairs with Pectasol-C
Innovita Vagus Nerve	Per form code	Valsalva 1.24, vagal impairment	Daily
Ion Gut Health	Per label	HDL 38, gut-liver axis	Daily
D-Ribose	Per label	Alpha/Theta imbalance, Energy 11	Cellular energy
Mito Synergy	Per label	Ceruloplasmin 31.3 (copper dysregulation)	Daily
L-Ornithine L-Aspartate	Per label	Uric Acid 700	Aldehyde pathway

Weeks 3–4

Supplement	Indication
Epi Pineal (Innovita)	Melatonin/pineal
Silent Night patch (LifeWave)	Melatonin — apply nightly
X39 patch (LifeWave)	Stem cell / UB Rates — 12 hr on / 12 hr off
Glutathione patch (LifeWave)	Detox / spike protein
Adipothin	Leptin resistance
Hypothala (Innovita)	MSH/pituitary support — working toward Cyto Lower gate
Fatty 15 (C15)	Cell membrane rebuilding (age 76)
Rejuvenation H2	Mitochondrial support

Delayed (When Gate Conditions Met)

- **Apex:** ONLY once VCS shows improvement. Do NOT start at VCS 2/32 with eGFR 56 — redistribution reaction risk.
- **Vitamin D maintenance:** After 4–6 weeks at 10,000 IU, retest. If >60: reduce to 4,000 IU maintenance + K2 800 mcg.

Five Levers Status

Lever	Status	Protocol
Melatonin	🔴 Depleted (Energy 11, Delta 80%)	Pineal Support freq · Epi Pineal · Silent Night patch

Leptin	🔴 Severe resistance (all organs red, Delta 80%)	Leptin Resist freq · Adipothin · Leptin Rules Diet
MSH	🔴 Almost certainly <8 (VCS 2/32) — **GATES CYTO LOWER**	Pit A Support + Pars Intermedia freqs · Hypothala · sunlight
Vitamin D	🟡 Suboptimal (41.9 — target 60–80)	10,000 IU D3 + K2 × 4–6 wk → 4,000 IU maintenance
UB Rates	⚪ Not activated	X39 patch — 12 hr on / 12 hr off daily

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Key Clinical Concerns

1. Persistent Spike Protein Burden (11,050 U/mL)

This is likely the primary driver of Denis's multi-system collapse. Spike protein at this level causes endothelial damage (Heart 2%, Blood Vessels 6%), mitochondrial dysfunction (all organs red, CRP 15.89), vagus nerve damage (Valsalva 1.24, PNS -3.5), blood-brain barrier disruption (Delta 80%, VCS 2/32), clotting cascade activation (hematocrit 51.3), and immune dysregulation perpetuating all of the above. Augmented NAC 3/day × 5–6 months + Nattokinase + Virus Recovery frequency + Cell Synergy. **This is not optional.**

2. Cardiovascular Crisis

Heart at 2% · hs-CRP 15.89 (16× high-risk threshold) · LDL 214 · HDL 38 · TG 175 · Homocysteine 18.4 · Hematocrit 51.3 · Absent vascular compensation · Blood Vessels 6%. This combination in a 76-year-old represents acute cardiovascular event risk. Serculate + CoQ10 + Heart Health are urgent.

3. Chronic Kidney Disease (eGFR 56 — Stage 3a)

eGFR 56 limits detox capacity. Every mobilized toxin must exit through kidneys at 56% capacity. This is why Apex is delayed. Kidney Repair/Vitality + Kidney Clear + Pectasol-C are essential. Hydration must be emphasized (hematocrit 51.3 may reflect dehydration).

4. Autonomic Nervous System Collapse

SNS switched · Vagal tone severely impaired (Valsalva 1.24, E/I 1.01) · PSNS -3.5 · No vascular compensation · Deep breathing = zero PNS recovery. The autonomic system cannot regulate. SNS Balance 35@709 must be first every session.

5. Biotoxic Illness (VCS 2/32)

VCS 2/32 is not borderline — it is profound. Indicates massive biotoxin load (mold, CIRS, Lyme, spike), severe neurological degeneration, or both. Delta 80% waking corroborates CNS suppression. Environmental assessment (home mold, water quality) is warranted.

Monitoring Recommendations

2-Week In-Office Recheck

- Urinalysis pH: look for rise from 5.08 toward >5.5
- VCS retest: any improvement from 2/32 — gate for adding Apex

- Supplement tolerance: 76yo on large stack — monitor GI tolerance, adjust timing

4-Week In-Office + Labs

- Full DePulse, HRV, Ortho, Valsalva, UA, VCS repeat
- Priority: Heart movement above 5%, PSNS trend, Delta brainwave reduction

6-Week LabCorp

Marker	Current	Target
hs-CRP	15.89	<5.0 (interim), <1.0 (long-term)
25-OH Vitamin D	41.9	>60
Creatinine / eGFR	1.32 / 56	Stable or improving
Ferritin	389	Trending <336
Homocysteine	18.4	<14.5
SARS-CoV-2 Spike Ab	11,050	Declining trend
Hematocrit	51.3	<50
MSH (ADD)	Unknown	**>8 — gates Cyto Lower**
RBC CoQ10 (ADD)	Unknown	>1.65

Red Flag Thresholds — Escalate Immediately

- eGFR <45 → reduce detox load immediately
- hs-CRP >20 → assess for acute inflammatory event / cardiac event
- Hematocrit >54 → phlebotomy consideration
- Chest pain, dyspnea, syncope → ER referral (Heart 2%)
- pH <5.0 → medical emergency

PATIENT GD — Geraldine DeMartino {#patient-gd}

Female | Age ~72 | DOB: 05/06/1951 Test Date: 05/02/2024 **No LabCorp blood panel available**

Urgency Rating: 5 / 5 — MAXIMUM URGENCY

GD presents with the most catastrophic bioenergetic profile in the patient cohort. Every single organ system registers at 1% on D-Pulse. Brain activity is 0%. Physiological resources volume is 4 out of a normal range of 150–600 — representing approximately 0.7% of minimum normal capacity. Stress index is 18.76× the upper limit of normal. She has active cardiac arrhythmias (bigeminy and trigeminy), a urine pH of 4.89 indicating extreme systemic acidosis, a VCS score of 3/32 indicating severe biotoxic burden, and a System Energy of 13 representing maximum energetic debt. This is not a patient with isolated findings — this is a patient in whom every measurable system has collapsed to floor values simultaneously.

Diagnostic Data Summary

DePulse+ — CATASTROPHIC

Metric	Value	Status
ALL organs	**1%**	🔴 FLOOR VALUE — every system
Brain activity	**0%**	🔴 ZERO
Immunity	**0%**	🔴 ZERO
Stress index	**1,876**	🔴 (normal 10–100)
Vegetative balance	**3,752**	🔴 (normal 35–140)
Physiological resources	**4**	🔴 (normal 150–600)
All vertebral columns	**1%**	🔴 FLOOR

HRV / NervXpress

- **System Energy:** 13 (WORST possible — maximum energetic debt)
- **Stress Response:** 1 (Best — paradoxical — see Paradox Analysis)
- **Brainwaves:** Delta 1% · Theta 7% · Alpha 18% · **Beta 50%** · **Gamma 24%**
- Beta + Gamma combined = **74%** — extreme hyperactivated/hypervigilant state
- **Beta > Alpha = deal breaker pattern → CPP frequency**

Orthostatic Test

- Physical Fitness Level: **13/1** (worst physiological functioning, highest adaptability — paradoxical)
- Chronotropic reaction: 0.48 · Compensation response: 0.84 · Ratio: 1.95
- Single extrasystoles noted
- PSNS: **-4.0** · SNS: +1.5
- POV supine=1, upright=1, Index of discrepancy=-3
- Machine conclusion: *"VASCULAR COMPENSATION DECREASES SHARPLY IN RESPONSE TO ORTHOTEST WHILE CHRONOTROPIC REACTION IS WITHIN NORMAL PARAMETERS"*

Valsalva Maneuver

- Valsalva Ratio: **2.02** (Athletic level — >1.7 — see Paradox Analysis)
- E/I Ratio: 1.01
- **Arrhythmias: TRIGEMINY, BIGEMINY, Single extrasystoles** — all documented
- PNS: sharply decreased upright; SNS: sharply increased with deep breathing

Urinalysis + Visual Contrast Scale

- **pH: 4.89** (EXTREME acidity — most acidic of all 5 patients)
- Uric Acid: 300 · Salinity: 300 · Zinc: 0.5 (very low) · Ascorbate: 0.6 (very low, depleted Vitamin C)
- **VCS: 3/32** — CRITICAL FAIL (passing = 24+/32)

Paradoxical Patterns — Clinical Explanation

These three paradoxes are **arrhythmia artifacts and HPA exhaustion signatures**, not signs of preserved function. Treat every system as critically compromised.

Paradox 1: System Energy 13 (Worst) + Stress Response 1 (Best)

This is HPA axis exhaustion — the body has used everything. Like a fire alarm with dead batteries, the system no longer has energy to mount a stress response even though the house is burning. The favorable Stress Response score does NOT mean GD is calm. It means the stress response mechanism has burned out. Classic Hans Selye "exhaustion phase."

Paradox 2: Valsalva Ratio 2.02 (Athletic) + PSNS -4.0 (Deeply Negative)

A Valsalva ratio above 1.7 normally indicates robust vagal tone. Here, it is **artifactual** — the bigeminy and trigeminy create alternating R-R intervals that inflate the Valsalva ratio mathematically **without reflecting true vagal tone**. The arrhythmias are mimicking athletic heart rate variability. GD is NOT cardiovascularly fit. The arrhythmia pattern is masquerading as vagal tone.

Paradox 3: Physical Fitness 13 (Worst) + Adaptability 1 (Highest)

Same mechanism — the "adaptability" score is inflated by arrhythmic R-R interval variability. The ortho-test conclusion confirms the body CANNOT adapt to postural change despite the score. Vascular compensation decreases sharply.

Deal Breakers Triggered — All 7

Deal Breaker #1: SNS SWITCHED

Evidence: PSNS -4.0, SNS +1.5, both dots upper-left quadrant. Without correcting autonomic tone, no other protocol will hold.

- **Frequency:** SNS Balance 35@709
- **Priority:** HIGHEST

Deal Breaker #2: BETA > ALPHA (CPP Pattern)

Evidence: Beta 50% vs Alpha 18% — 2.78× ratio. Beta + Gamma = 74%. Near-absence of Delta (1%) means deep restorative sleep is essentially absent. This hypervigilant, stress-locked brain cannot downregulate.

- **Frequency:** CPP
- *Note: Theta/Alpha deal breaker is NOT triggered (Theta 7% < Alpha 18%), but Beta > Alpha IS triggered*

Deal Breaker #3: PNS NEGATIVE ZONE

Evidence: PSNS -4.0. E/I ratio 1.01. Deep breathing produces essentially no PNS improvement.

- **Frequency:** PNS Support

Deal Breaker #4: VAGUS NERVE DYSFUNCTION

Evidence: PSNS -4.0; bigeminy/trigeminy during Valsalva; E/I 1.01; PNS sharply decreased upright; SNS sharply increases with deep breathing (paradoxical). The vagal pathway is not merely weak — it is functionally disrupted.

- **Frequencies:** Vagus Support · Vagus Balance · Vagus Trauma · **Medulla Support** (for arrhythmias)
- **Supplement:** Innovita Vagus Nerve form code

Deal Breaker #5: LOW HEART ON D-PULSE (1%)

Evidence: Heart 1% while active arrhythmias (bigeminy/trigeminy) are documented. Electrically unstable and energetically depleted simultaneously.

- **Frequency:** Heart Health
- **Supplements:** Serculate, CoQ10

Deal Breaker #6: LOW pH (4.89) — EXTREME ACIDOSIS

Evidence: pH 4.89 — approximately 40× more acidic than the mandatory Tri-Salts threshold of 6.2 (logarithmic scale). Cellular enzyme function impaired. Electron spin inversion biochemical environment.

- **Supplements:** Cell Synergy (2–3 scoops) + Tri-Salts (MANDATORY)
- **Frequency:** Terrain

Deal Breaker #7: VCS FAILED (3/32)

Evidence: VCS 3/32 — worst possible category. Indicates massive biotoxin accumulation. Virtually guarantees critically low MSH.

- **Frequency:** Biotoxin
- **Supplements:** Pectasol-C (immediate); Apex — DELAYED until VCS improving

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Frequency Protocols — Complete List

Phase 1: Deal Breakers (Sessions 1–4)

Priority	Frequency	Indication
1	**SNS Balance 35@709**	SNS switched — must precede everything
2	**CPP**	Beta 50% > Alpha 18%
3	**PNS Support**	PSNS -4.0 negative zone
4	**Vagus Support**	Vagal pathway disruption
5	**Vagus Balance**	Complement to Vagus Support
6	**Vagus Trauma**	Chronic vagal disruption — deep repair
7	**Medulla Support**	Bigeminy/trigeminy cardiac rhythm center
8	**Heart Health**	Heart 1%, active arrhythmias
9	**Terrain**	pH 4.89, all systems 1%
10	**Biotoxin**	VCS 3/32 — initiate clearance

Phase 2: Organ/System Support (Sessions 5–10)

Frequency	Indication
Pineal Support	System Energy 13, Delta 1% — melatonin
Lung Support	Lungs 1%
Kidney Repair/Vitality	Kidneys 1%
Bladder Support	Bladder 1%

Sacral Plexus	Sacral column 1%
Cervical/Thoracic/Lumbar Support	All vertebral columns 1%
Leptin Resist	Energy 13, organ collapse — Five Levers
Pit A Support + Pars Intermedia	VCS 3/32 → MSH restoration — **gates Cyto Lower**

Phase 3: Condition-Specific (Sessions 10+)

Frequency	Indication
Concussion	POTS-like pattern (sharp vascular compensation decrease)
Vein Vitality	Vascular integrity support

HARD GATE: Cyto Lower

Required for all-organs-1% pattern (electron spin inversion, Krebs reversal) but CANNOT run until MSH confirmed >8. At VCS 3/32, MSH is presumed critically low. Pit A Support + Pars Intermedia + Hypothala must come first. MSH must be lab-confirmed. Running Cyto Lower with suppressed MSH risks a cytokine storm the body cannot manage. This is weeks to months away. Duration when approved: 1 hr 45 min.

Arrhythmia Considerations (Bigeminy / Trigeminy)

Frequency approach: Medulla Support (cardiac rhythm centers) + Vagus Support + Heart Health address the bioelectric and neurological origin of rhythm disturbance. SNS Balance must precede arrhythmia protocols — SNS overdrive is destabilizing cardiac rhythm.

Safety monitoring:

- Track arrhythmia pattern at every session
- If bigeminy progresses to sustained runs, trigeminy increases, or new patterns emerge (couplets, triplets, non-sustained VT) → pause cardiac protocols → conventional cardiology referral
- Patient must report palpitations >30 seconds, lightheadedness, near-syncope, or syncope immediately

Contraindications:

- Do NOT run aggressive detox (Cyto Lower, Apex) until arrhythmias have stabilized — electrolyte shifts from detox can exacerbate arrhythmias
- Do NOT push cold thermogenesis (Leptin Rules) — SNS overdrive contraindication

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Supplementation Stack

Day 1 — Immediate

Supplement	Dose	Indication
Cell Synergy	**2–3 scoops**	pH 4.89, terrain collapse, systemic acidosis

Tri-Salts	Per label (mandatory)	pH 4.89 — far below 6.2 mandatory threshold
Pectasol-C	Per label (~5g/day)	VCS 3/32 — gentle biotoxin binding, safe to start immediately
Serculate	Per label	Heart 1%, arrhythmias, circulatory support
CoQ10 (ubiquinol)	200–400 mg	Heart 1% — target RBC CoQ10 >1.65
Innovita Vagus Nerve	Per form code	PSNS -4.0, vagal pathway disrupted

First 2 Weeks

Supplement	Indication
Epi Pineal (Innovita)	System Energy 13, Delta 1% — melatonin
Silent Night patch	Melatonin — apply per LifeWave protocol
X39 patch	12 hr on / 12 hr off — UB Rates
Glutathione patch	Detox support, VCS 3/32
Hypothala (Innovita)	MSH/hypothalamic support (presumed critically low)
Adipothin	Leptin resistance (Energy 13, universal organ collapse)
Rejuvenation H2	Mitochondrial support, all systems collapsed

Ongoing

Supplement	Indication
Fatty 15 (C15)	Cell membrane rebuilding — all membranes compromised at 1%
Ion Gut Health	Gut integrity at pH 4.89
D-Ribose	5g 2–3×/day — cellular energy substrate
Kidney Clear	Kidneys 1%, renal detox support

Delayed (Gate Conditions Required)

- **Apex:** Only once VCS is IMPROVING — not at 3/32. Add Apex too early risks herxheimer reaction with physiological resources of 4/600.
- **Integra Cell:** Obtain CRP first. If elevated, add.
- **Vitamin D3 (10,000 IU + K2):** Obtain 25-OH Vitamin D level first. If deficient: 10,000 IU + K2 MK-7. If <20 ng/mL: consider loading dose.

Five Levers Status

Lever	Status	Action

Melatonin	🔴 Critically depleted (Energy 13, Delta 1%)	Epi Pineal + Silent Night + Pineal Support
Leptin	🔴 Severely disrupted, post-menopausal compounding	Adipothin + Leptin Resist
MSH	🔴 Presumed critically low (VCS 3/32) — **GATES CYTO LOWER**	Pit A + Pars Intermedia + Hypothala + morning sunlight
Vitamin D	❓ Unknown — no labs	Obtain 25-OH Vitamin D URGENTLY
UB Rates	⚪ Not activated	X39 12 hr on/off — begin immediately

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Urgent Labs to Obtain (No LabCorp Yet)

Test	Urgency	Why
MSH	IMMEDIATE	Gates Cyto Lower
25-OH Vitamin D	IMMEDIATE	Five Levers
CMP (electrolytes, kidney/liver)	IMMEDIATE	Arrhythmias — potassium and magnesium critical
RBC Magnesium	IMMEDIATE	Low Mg is pro-arrhythmic
CBC with Differential	IMMEDIATE	0% immunity on DePulse — baseline immune status
CRP	Urgent	Systemic inflammation baseline
RBC CoQ10	Within 1 week	Cardiac supplement dosing guide
Cortisol (AM + PM or 4-point salivary)	Within 2 weeks	HPA exhaustion pattern — paradox #1
Leptin (fasting, AM)	Within 2 weeks	Lever #2 confirmation
Thyroid panel (TSH, Free T3/T4, Reverse T3)	Within 2 weeks	Universal organ suppression

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PATIENT GG — Gregory Geiger {#patient-gg}

Male | Age ~69 | DOB: 08/26/1955 Test Date: 08/12/2025 **No LabCorp blood panel available**

Urgency Rating: 5 / 5 — CRITICAL

GG presents with total bioelectric collapse across all organ systems. Every major system scored 1-7%, with Heart, Kidneys, Bladder, Lungs, Stomach, and Gallbladder all at 1%. This is not a partial system decline — it is total bioelectric collapse. Additionally, GG exhibits a **paradoxical deep breathing response** — a hallmark

of Vagus Trauma that fundamentally changes the protocol approach. Combined with proteinuria 3.0 (significant kidney pathology even without blood labs), VCS 8/32, and PSNS -3.0, this is a patient in critical multi-system failure.

Diagnostic Data Summary

DePulse+

Organ	Score
Heart	1% 🔴
Kidneys	1% 🔴
Bladder	1% 🔴
Lungs	1% 🔴
Stomach	1% 🔴
Gallbladder	1% 🔴
All other organs	1–7% 🔴

HRV / NervXpress

- **System Energy:** 11 (Energetic Debt)
- **Stress Response:** 7 (Worst)
- **Brainwaves:** Delta 61% · Theta 18% · Alpha 10% · Beta 7% · Gamma 4%
- **Theta 18% > Alpha 10%** = deal breaker

Orthostatic Test

- Physical Fitness Level: **11/7**
- PSNS: **-3.0** · SNS: +1.5
- Vascular compensation: **ABSENT** · Chronotropic reaction: 0.90 (decreases)
- POV supine=1, upright=4, Index of discrepancy=-1

Valsalva Maneuver

- Valsalva Ratio: **1.32** (bottom of average — 1.3–1.7)
- E/I Ratio: 1.04
- **Deep breathing → NEGATIVE changes in PNS** (paradoxical — deep breathing WORSENS PNS)

Urinalysis + Visual Contrast Scale

- **pH: 5.79** (acidic, below 6.5 threshold; above 6.2 so Tri-Salts not yet mandatory)
- **Protein: 3.0** (significant proteinuria — glomerular filtration barrier failure)
- Uric Acid: 300 (elevated) · Magnesium: 20 · Ascorbate: 2.8
- **VCS: 8/32** — CRITICAL FAIL

Special Focus: Paradoxical Deep Breathing Response

What Is Happening

In a healthy individual, deep breathing activates the Vagus nerve, which increases PNS tone. GG's test shows the opposite: when he performs deep breathing, PNS activity **decreases** — it gets worse.

Clinical Interpretation

This is a hallmark of **Vagus Nerve Trauma**, distinct from Vagus deficiency:

- **Vagus deficiency** (weak but functional): Deep breathing still improves PNS, just insufficiently. Treat with Vagus Support.
- **Vagus trauma** (dysfunctional — GG's pattern): The nerve has been damaged or rewired by physical trauma, chronic biotoxic insult, or prolonged autonomic stress. Stimulation produces the **wrong output**. Deep breathing is actively harmful.

Protocol Changes

1. **Vagus Trauma frequency MUST precede Vagus Support.** Running Vagus Support on a traumatized Vagus amplifies the dysfunction.
2. **Do NOT prescribe breathwork, meditation, or vagal maneuvers to GG** until Vagus Trauma has been run and PNS response to deep breathing is retested.
3. **Sequence:** Vagus Trauma (Phase 1) → retest at Week 4 → if deep breathing now improves PNS, transition to Vagus Support → then Vagus Balance.
4. The E/I ratio of 1.04 (barely above unity) confirms vagal output is essentially absent.
5. **Investigate history:** Concussion, whiplash, cervical injury, mold exposure, Lyme disease — any could explain Vagus Trauma pattern.

Special Focus: Proteinuria 3.0

What It Means

Protein 3.0 in urine indicates failure of the glomerular filtration barrier — the kidneys are leaking protein that should remain in the blood. Combined with Kidneys at 1% on DePulse, this is convergent evidence of serious kidney pathology. Without blood labs, GFR is unknown — GG could be at CKD Stage 3 or 4.

Treatment Implications

- Kidney support is **URGENT, not optional**, even without blood panel confirmation
- **CMP with BUN, creatinine, GFR is the most urgent lab** — must be obtained immediately
- Pectasol-C serves double duty: kidney clearance + biotoxin binding
- Monitor urine protein at every visit — protein trending down = kidneys recovering
- pH 5.79 is partly a consequence of kidney failure (kidneys unable to excrete acid/reabsorb bicarbonate)

Deal Breakers Triggered — All 7

Deal Breaker #1: SNS SWITCHED

Evidence: PSNS -3.0, SNS +1.5, vascular compensation absent. SNS Balance must precede all others.

- **Frequency:** SNS Balance 35@709

Deal Breaker #2: VAGUS NERVE DYSFUNCTION (TRAUMA PATTERN)

Evidence: Deep breathing WORSENS PNS — this is the defining finding. E/I 1.04. PSNS -3.0.

- **Primary frequency: Vagus Trauma** (NOT Vagus Support — this distinction is critical)
- After retest shows deep breathing improving PNS: transition to Vagus Support → Vagus Balance
- **Supplement:** Innovita Vagus Nerve

Deal Breaker #3: ALPHA/THETA IMBALANCE

Evidence: Theta 18% > Alpha 10%. Delta 61% (approaching 70% low-DC threshold — treat as low direct current).

- **Frequency:** Alpha Theta
- **Supplements:** Cell Synergy (mandatory), D-Ribose

Deal Breaker #4: PNS NEGATIVE ZONE

Evidence: PSNS -3.0, Valsalva 1.32 (bottom of average), paradoxical deep breathing.

- **Frequency:** PNS Support

Deal Breaker #5: LOW HEART ON D-PULSE (1%)

Evidence: Heart 1%. Chronotropic reaction decreases. POV supine=1, upright=4 — orthostatic cardiac instability.

- **Frequency:** Heart Health
- **Supplements:** Serculate, CoQ10

Deal Breaker #6: LOW pH (5.79)

Evidence: pH 5.79 — below 6.5 deal-breaker threshold. Above 6.2 so Tri-Salts not yet mandatory. Kidney failure is contributing to acidosis (Kidneys 1%). If pH doesn't respond to Cell Synergy by Week 2 → add Tri-Salts + Terrain.

- **Supplement:** Cell Synergy (1–2 scoops, titrate to 2–3)
- **Frequency:** Terrain (if pH doesn't respond by Week 2)

Deal Breaker #7: VCS FAILED (8/32)

Evidence: VCS 8/32 — 75% of visual contrast sensitivity lost. Strongly suggests MSH suppressed.

- **Frequency:** Biotoxin
- **Supplements:** Pectasol-C (immediate); Apex (delayed)

Frequency Protocols — Complete List

Phase 1 (Sessions 1–4)

Priority	Frequency	Evidence
1	**SNS Balance 35@709**	PSNS -3.0, SNS +1.5, absent vascular compensation
2	**Vagus Trauma**	Deep breathing WORSENS PNS — trauma pattern confirmed
3	**Alpha Theta**	Theta 18% > Alpha 10%
4	**PNS Support**	PSNS -3.0 negative
5	**Heart Health**	Heart 1%, ChMR 0.90

6	**Biotoxin**	VCS 8/32
7	**Terrain**	Conditional — if pH doesn't respond to Cell Synergy

Phase 2 (Weeks 2–4)

Frequency	Indication
Kidney Repair/Vitality	Kidneys 1% + proteinuria 3.0 + Uric Acid 300
Vagus Support	Add AFTER deep breathing retest normalizes (Week 4)
Vagus Balance	Add after Vagus Support shows improvement
Lung Support	Lungs 1% + VCS 8/32 (biotoxin/respiratory link)
Concussion	POTS-like pattern, absent vascular compensation
Vein Vitality	Absent vascular compensation, POV 1→4
Pineal Support	Energy 11, Delta 61% — melatonin
Bladder / Liver / GB / Digestive Support	Each at 1% on DePulse

Phase 3 (Weeks 3–6)

Frequency	Indication
Aldehyde Detox	Uric Acid 300
Leptin Resist	Energy 11, all organs 1%
Pit A Support + Pars Intermedia	VCS 8/32 → MSH restoration

Key Decision Point: Week 4

Retest deep breathing PNS response within HRV. If PNS now **improves** with deep breathing → transition from Vagus Trauma to Vagus Support. If still paradoxical → continue Vagus Trauma.

GATE: Cyto Lower

Obtain MSH. If >8: add Cyto Lower. If <8: continue Biotoxin + Pit A + Pars Intermedia and retest at Week 8.

Supplementation Stack

Day 1

Supplement	Dose	Indication
Cell Synergy	1–2 scoops (titrate to 2–3)	pH 5.79, Delta 61%, Energy 11
CoQ10	Standard (target RBC >1.65)	Heart 1%
Serculate	Per label	Heart 1%, POTS pattern

Pectasol-C	Per label	VCS 8/32, Kidneys 1%, Uric Acid 300 — triple duty
Innovita Vagus Nerve	Per label	Vagus Trauma pattern
D-Ribose	Per label	Energy 11, Alpha/Theta imbalance

First 2 Weeks

Supplement	Indication
Epi Pineal	Energy 11, Delta 61%
Silent Night patch	Melatonin
X39 patch	12 hr on/off — UB Rates
Glutathione patch	Detox, VCS 8/32
Kidney Clear	Kidneys 1%, Protein 3.0
L-Ornithine L-Aspartate	Uric Acid 300
Hypothala	VCS 8/32, likely suppressed MSH

Ongoing

Adipothin · Ion Gut Health · Rejuvenation H2 · Fatty 15 (C15)

Conditional

- **Tri-Salts:** Add if pH doesn't improve above 6.2 by Week 2
- **Apex:** Only once VCS trending upward
- **Vitamin D3 + K2:** Obtain 25-OH Vitamin D first

Five Levers Status

Lever	Status	Action
Melatonin	🔴 Depleted (Energy 11, Delta 61%)	Epi Pineal + Silent Night + Pineal Support
Leptin	🔴 Severely suppressed	Adipothin + Leptin Resist
MSH	🔴 Likely suppressed (VCS 8/32) — **GATES CYTO LOWER**	Pit A + Pars Intermedia + Hypothala + morning sunlight
Vitamin D	❓ Unknown — no labs	Obtain 25-OH Vitamin D urgently
UB Rates	⚪ Not activated	X39 12 hr on/off — activate immediately

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Urgent Labs to Obtain

Test	Urgency	Why
CMP (BUN, Creatinine, GFR)	**IMMEDIATE**	Proteinuria 3.0 + Kidneys 1% — may be CKD 3 or 4
MSH	IMMEDIATE	Gates Cyto Lower
25-OH Vitamin D	IMMEDIATE	Five Levers — Lever 4
CBC	High	0% immunity on DePulse baseline
Albumin + Total Protein (serum)	High	Quantify protein loss
Uric Acid (serum)	Within 1 week	Correlate with UA 300
RBC CoQ10	Within 1 week	Cardiac dosing guide
CRP	Within 2 weeks	Inflammation baseline
Lipid panel	Within 2 weeks	Cardiovascular risk at 69
TSH + Free T3/T4	Within 2 weeks	Thyroid — common contributor
Cortisol (AM)	Within 2 weeks	Adrenal status
Leptin	Within 2 weeks	Lever 2 confirmation

PATIENT JW — Jennifer Weber {#patient-jw}

Female | Age ~52 | DOB: 06/05/1973 Test Date: 02/24/2026 **No LabCorp blood panel available**

Urgency Rating: 2.5 / 5

JW presents with the **mildest profile of the five-patient cohort** — but urgency is NOT low. Five deal breakers remain active, Stress Response is 7 (worst, same as catastrophic patients), Vegetative Balance is 332 (2.4× upper limit of normal), and PNS is -2.5 with no deep-breathing recovery. She is the same house on fire as the severe patients, just earlier in the fire.

Favorable features distinguishing JW:

- **Passing VCS (25/32)** — only patient to pass; biotoxin pathway not critically suppressed; MSH likely borderline, not critically depleted
- **No catastrophic organ scores** — worst is Sacrum at 13%; everything else 28–43% (yellow zone, not red)
- **Physiological Resources: 110** — still has reserves (others have 4–34)
- **System Energy: 10** — beginning of debt, not deep in it

- **No bigeminy/trigeminy** — only single extrasystoles

Diagnostic Data Summary

DePulse+

Metric	Value
Stress index	173 (normal 10–100) 🟡
Vegetative balance	332 (normal 35–140) 🟡
Brain activity	27% 🟡
Immunity	42% 🟡
Physiological resources	110 (normal 150–600) 🟡
Sacrum	**13%** 🟠 (outlier — significantly below others)
Heart	32% 🟡
Kidneys	30% 🟡
Bladder	28% 🟡
All others	28–43% 🟡 (yellow/stress zone)

HRV / NervXpress

- **System Energy:** 10 (beginning of Energetic Debt)
- **Stress Response:** 7 (Worst — same as catastrophic patients)
- **Brainwaves:** Delta 47% · Theta 31% · Alpha 10% · Beta 10% · Gamma 2%
- **Theta 31% > Alpha 10%** = deal breaker (3:1 ratio)
- **Quadrant: LOWER-LEFT** — Energetic Imbalance (both SNS and PNS depleted/negative)

Orthostatic Test

- Physical Fitness Level: **10/7** (moderately reduced)
- PSNS: **-2.5** · SNS: **-1.0** (BOTH negative — lower-left pattern)
- Chronotropic reaction: 0.82 · Vascular compensation absent
- **POV supine=10** (unusual — most patients = 1), upright=1
- Index of discrepancy: +1 (positive — unusual)
- Single extrasystoles noted

Valsalva Maneuver

- Valsalva Ratio: **1.29** (borderline — just below 1.3 threshold)
- E/I Ratio: 1.07
- Mild sympathetic hyper-regulation noted
- No PNS improvement with deep breathing (but does NOT worsen — important distinction from GG)

Urinalysis + Visual Contrast Scale

- **pH: 6.43** (borderline — below 6.5 threshold, but above 6.2 Tri-Salts trigger)
- Specific Gravity: **1.005** (very low — dilute urine)
- Uric Acid: 100

- **VCS: 25/32 — PASSING** (only patient in cohort to pass)

Lower-Left Quadrant Pattern — Clinical Analysis

What This Pattern Means (vs. Upper-Left "SNS Switched")

Upper-Left quadrant (DH, GD, TH, GG): High SNS, negative PNS. The sympathetic system is switched on and dominating. The body is misdirecting energy. Treatment: correct the switch.

Lower-Left quadrant (JW — unique): BOTH SNS and PNS are negative/depleted. The body hasn't misdirected energy — it has run out of energy in both directions. This is total autonomic depletion, not misdirection.

Clinical Implications

1. **Energy restoration is the primary strategy** — not "switching" one arm. Mitochondrial support (Cell Synergy, D-Ribose, Rejuvenation H2) is more central to JW than for SNS-switched patients.
2. **Recovery may be more gradual but more stable** — as cellular energy returns, both ANS arms come up together. Less risk of autonomic instability during treatment (no sudden "switching back on").
3. **POV supine=10** (unusual) suggests JW's parasympathetic system retains residual variability in resting state — it is depleted but not completely absent. This supports better prognosis.
4. **SNS Balance** is still appropriate but for rebalancing, not for "turning on" a switched-off arm.

Deal Breakers Triggered — 5 of 7

Deal Breaker #1: ALPHA/THETA IMBALANCE (Highest Priority)

Evidence: Theta 31% >> Alpha 10% — 3:1 ratio. Delta 47% approaching 70% low-DC threshold. The brain cannot produce organized Alpha rhythms required for autonomic regulation and cognitive function.

- **Frequency:** Alpha Theta (HIGHEST priority — upstream driver)
- **Supplements:** Cell Synergy (1 scoop — standard dose for mild case), D-Ribose

Deal Breaker #2: LOWER-LEFT ANS DEPLETION

Evidence: SNS -1.0, PSNS -2.5 — both negative. Lower-left quadrant. Total autonomic depletion.

- **Frequency:** SNS Balance 35@709 (rebalancing, not switching); PNS Support

Deal Breaker #3: PNS NEGATIVE ZONE

Evidence: PSNS -2.5, Valsalva 1.29 (<1.3), no PNS improvement with deep breathing (but doesn't worsen).

- **Frequency:** PNS Support

Deal Breaker #4: VAGUS DYSFUNCTION (Deficiency, NOT Trauma)

Evidence: PSNS -2.5, Valsalva 1.29, no deep-breathing PNS improvement (but does NOT worsen). This is depleted vagal tone, not a traumatized nerve — the critical distinction from GG.

- **Frequency: Vagus Support** (NOT Vagus Trauma — deep breathing doesn't worsen)
- **Supplement:** Innovita Vagus Nerve

Deal Breaker #5: BORDERLINE LOW pH (6.43)

Evidence: pH 6.43 — below the 6.5 deal-breaker threshold (just barely). Above 6.2 — Tri-Salts not yet mandatory.

- **Supplement:** Cell Synergy (1 scoop); recheck pH in 2 weeks
- **Frequency:** Terrain ONLY if pH doesn't respond to Cell Synergy by Week 2

NOT Triggered (Favorable Prognostic Indicators)

- **VCS:** 25/32 = PASSING — biotoxin pathway not critically suppressed
- **Heart deal breaker:** Heart 32% — yellow zone but NOT below 5% critical threshold
- **Bigeminy/Trigeminy:** Not present — only single extrasystoles

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Frequency Protocols — Complete List

Phase 1: Deal Breakers + Primary Autonomic Support (Weeks 1–4)

Priority	Frequency	Evidence
1	**Alpha Theta**	Theta 31% >> Alpha 10% — upstream brain driver
2	**SNS Balance 35@709**	Both arms depleted, lower-left quadrant
3	**PNS Support**	PSNS -2.5, Valsalva 1.29
4	**Vagus Support**	No PNS recovery with deep breathing — deficiency, not trauma
5	**Pineal Support**	Energy 10, beginning of energetic debt
Conditional	Terrain	Only if pH remains <6.5 after 2 weeks Cell Synergy

Phase 2: Organ Support (Weeks 3–6)

Frequency	Indication
Sacral Plexus	Sacrum 13% — outlier, significantly below others
Heart Health	Heart 32% (yellow — support, not deal-breaker)
Kidney Support	Kidneys 30%, Specific Gravity 1.005 (dilute urine)
Bladder Support	Bladder 28%
Leptin Resist	Energy 10, probable early leptin resistance

Phase 3: Optimization + Levers (Weeks 5–8+)

Frequency	Indication
Brain Activity Support	Brain 27%
Immune Support	Immunity 42%
Stress Index Regulation	Stress Index 173, Vegetative Balance 332

MSH Support (conditional)	Only if VCS declines below 24/32 on retest

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Supplementation Stack

Day 1

Supplement	Dose	Indication
Cell Synergy	**1 scoop** (standard — milder case)	Brainwave support, borderline pH 6.43, Delta 47%
D-Ribose	5g 2×/day	Cellular energy, Alpha/Theta imbalance
Epi Pineal (Innovita)	Per label	Energy 10, melatonin lever
Silent Night patch	Nightly	Melatonin support
X39 patch	12 hr on/off	UB Rates — Five Levers
Innovita Vagus Nerve	Per form code	PSNS -2.5, Valsalva 1.29

First 2 Weeks

Supplement	Indication
Adipothin	Probable early leptin resistance
Rejuvenation H2	Lower-left quadrant = total energy depletion
Ion Gut Health	pH 6.43, gut integrity
Hypothala	VCS 25/32 (barely passing) — pituitary axis optimization

Ongoing

Fatty 15 (C15) · Glutathione patch (optional)

Note: No Pectasol-C urgency (VCS passing). No Tri-Salts (pH above 6.2). 1-scoop Cell Synergy only. No aggressive detox protocols.

Five Levers Status

Lever	Status	Action
Melatonin	🟡 Depleted (Energy 10)	Epi Pineal + Silent Night + Pineal Support
Leptin	🟡 Probable early resistance	Adipothin + Leptin Resist
MSH	🟢 Borderline (VCS 25/32 — barely passing)	Hypothala; **monitor VCS closely** — if drops below 24, escalate
Vitamin D	❓ Unknown — no labs	Obtain 25-OH Vitamin D URGENTLY

UB Rates	⚪ Not activated	X39 12 hr on/off

Prognosis and Treatment Trajectory

Expected Recovery Timeline

JW has the **best prognosis of the five patients.**

- **Weeks 1–2:** Brainwave improvement (Theta/Alpha ratio narrowing). Energy may begin rising from 10 → 15–20. pH should respond to Cell Synergy.
- **Weeks 3–4:** Autonomic improvement — PSNS beginning to move toward positive. Organ scores starting to climb from yellow toward 50%+.
- **Weeks 5–8:** Energy 20–30+, PSNS approaching positive territory, organ scores 50%+, Sacrum climbing above 25%.
- **Weeks 8–12:** Target: Energy 30+, PSNS positive, Valsalva >1.3, organs 50–60%+, pH >6.5 stable, brainwaves normalizing (Alpha > Theta).

Early Warning Signs — Escalate If:

Warning	Action
VCS drops below 24/32	Add Pectasol-C immediately, reassess MSH
Energy drops below 7	Increase Cell Synergy to 2-scoop protocol
Any organ drops to single digits	Escalate that organ's frequency to Phase 1 priority
Extrasystoles → bigeminy/trigeminy	Cardiology referral
Delta exceeds 70%	Confirm low direct current; Cell Synergy critical
pH drops below 6.0	Add Terrain frequency immediately

Monitoring Recommendations

Week 1 Labs (Urgent)

25-OH Vitamin D · CMP (BUN, Creatinine, GFR — Specific Gravity 1.005 warrants kidney check) · CBC · Leptin · hsCRP · TSH/T3/T4 · Fasting Insulin + Glucose

Retest Schedule

- **Week 2:** UA (pH must be >6.5) · VCS (confirm stability ≥24/32)
- **Week 4:** Full DePulse + HRV + Ortho + Valsalva + UA + VCS
- **Week 8:** Full panel repeat + LabCorp
- **Week 12:** Full comprehensive reassessment

PATIENT TH — Tim Haskins {#patient-th}

Male | Age 53 | DOB: 05/14/1972 Test Dates: DePulse/HRV/UA 02/03/2026 · LabCorp 02/10/2026

Urgency Rating: 5 / 5 — MAXIMUM URGENCY

Tim Haskins presents the most medically complex and clinically urgent case in this cohort. The convergence of uncontrolled Type 2 Diabetes (HbA1c 9.6%), CKD Stage 3a (eGFR 50), hepatic injury (AST 57 — deal breaker), catastrophic autonomic collapse (PSNS -4.0, SNS +2.5), "BE MORE ATTENTIVE" triggered on **both** Orthostatic AND Valsalva, Heart at 1%, VCS 12/32, and pH 5.43 represents a multi-system crisis requiring immediate, aggressive, and carefully sequenced intervention.

Diagnostic Data Summary

DePulse+

Metric	Value
Stress index	**673** (normal 10–100) 🔴
Vegetative balance	**915** (normal 35–140) 🔴
Brain activity	5% 🔴
Immunity	7% 🔴
Physiological resources	**34** (normal 150–600) 🔴
Heart	**1%** 🔴
Sacrum	**1%** 🔴
Cervical vertebra	3% 🔴
All organs	1–11% 🔴

HRV / NervXpress

- **System Energy:** 11 (Energetic Debt)
- **Stress Response:** 7 (Worst)
- **Brainwaves:** Delta 56% · Theta 31% · Alpha 8% · Beta 3% · Gamma 2%
- **Theta 31% > Alpha 8%** = deal breaker (4:1 ratio)
- **Quadrant: UPPER-LEFT** — high SNS, deeply negative PNS

Orthostatic Test

- Physical Fitness Level: **11/7**
- PSNS: **-4.0** (worst in cohort) · SNS: **+2.5** (highest in cohort)
- Chronotropic reaction: 0.95 (decreases) · Ortho-Test Ratio: **0.99** (unusual — below 1.0)
- POV supine=1, upright=1, Index of discrepancy: **-3** (worst in cohort)
- ⚠️ **"BE MORE ATTENTIVE TO THIS PATIENT"** — machine warning

Valsalva Maneuver

- Valsalva Ratio: 1.42 (average)
- E/I Ratio: **1.02** (essentially flat)
- PNS: sharply decreased · SNS: significantly elevated
- ⚠️ **"BE MORE ATTENTIVE TO THIS PATIENT"** — machine warning (second time)

Urinalysis + Visual Contrast Scale

- **pH: 5.43** (critical — far below 6.5; far below 6.2 mandatory Tri-Salts threshold)
- **Urine Glucose: 3+** (glucose spilling — severe hyperglycemia confirmed)
- Salinity: 200 · Uric Acid: 100
- **VCS: 12/32** — CRITICAL FAIL

LabCorp Blood Panel (02/10/2026)

Marker	Value	Status
Glucose	**151**	🔴 HIGH (normal 70–100)
HbA1c	**9.6%**	🔴 VERY HIGH — uncontrolled T2D (>6.5% = diabetes, >9% = severe)
BUN	29	🟡 Upper end
Creatinine	**1.64**	🔴 HIGH (normal <1.17)
eGFR	**50**	🔴 CKD Stage 3a
Phosphorus	**4.3**	🟡 HIGH — kidneys not clearing phosphorus
LDH	**258**	🔴 HIGH (normal 122–222) — cellular injury
AST	**57**	🔴 HIGH (normal 0–40) — liver enzyme — **DEAL BREAKER**
ALT	44	🟡 Upper limit
HDL	37	🔴 LOW (normal >40)
hs-CRP	3.01	🔴 HIGH cardiovascular risk (>3.0)
Homocysteine	**17.6**	🔴 HIGH (normal 0–14.5)
Reverse T3	15.7	✅ Normal
Vitamin D (25-OH)	37.3	🟡 Suboptimal (target 60–80)
Ferritin	369	🟡 Elevated (normal male 24–336)
Urine Glucose	**3+ Abnormal**	🔴 CRITICAL — glucose spilling
Insulin	7.7	✅ Normal (but paradoxically low given glucose 151)
CO2	20	🟡 Low — metabolic acidosis indicator

"BE MORE ATTENTIVE" — Double Trigger Analysis

What the Double Trigger Means

Most patients who trigger "BE MORE ATTENTIVE" trigger it on one test. TH triggered it on **both Orthostatic AND Valsalva.**

Orthostatic trigger: The ANS cannot compensate for the simple act of standing up. ChMR 0.95 (decreases), Ortho Ratio 0.99 (below 1.0), PSNS -4.0, SNS +2.5 — the cardiovascular-autonomic axis is failing its most basic task.

Valsalva trigger: Despite Valsalva Ratio 1.42 (average), E/I Ratio 1.02 is essentially flat — vagal output is absent. The average Valsalva Ratio with absent vagal tone means the heart is maintaining ratio via sympathetic compensation alone — an unsustainable state.

Double trigger means:

1. Autonomic failure is **systemic, not situational** — present across multiple physiological challenges simultaneously
2. Both baroreceptor-mediated (Valsalva) AND postural-mediated (Ortho) pathways are failing
3. Syncope risk, arrhythmia risk, and sudden cardiovascular event risk are elevated
4. The patient's subjective symptom report may not reflect the severity — chronic SNS overdrive patients often normalize their symptoms

Protocol Changes Mandated by Double Trigger

- **Minimum 2× per week sessions** (ideally 3× per week for first 4 weeks)
- **DOUBLE DOSE Cell Synergy** — 4 scoops (standard 2 scoops × 2)
- Autonomic parameters must be reassessed at every session
- Patient education: compliance is not optional — it is urgent

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T2D / Metabolic Protocol (HbA1c 9.6%)

The Clinical Picture

TH presents the most severe T2D profile possible short of hospitalization. The paradox of **normal insulin (7.7) with glucose 151** is clinically significant: the pancreas should be producing elevated insulin to compensate for glucose 151. It cannot. This suggests the pancreas is approaching **beta-cell exhaustion** — the late stage of T2D where insulin production itself begins to fail.

Urine Glucose 3+ confirms blood glucose is consistently exceeding the renal threshold (~180 mg/dL). The kidneys, already at CKD Stage 3a, are under additional osmotic/nephrotoxic stress from glucose filtration.

CO2 of 20 (low-normal) indicates metabolic acidosis partially driven by the diabetic state — not full DKA, but chronic subclinical acidosis. This aligns with pH 5.43.

T2D Frequencies

Frequency	Indication
Insulin Resist	Primary — insulin resistance
Pancreas T2D	Pancreatic beta-cell support
SIBO (nightly)	Per master protocol — gut-metabolic axis, T2D

T2D Supplements

- Cell Synergy (double dose) — mitochondrial function critical for glucose metabolism
- Rejuvenation H2 — oxidative stress from chronic hyperglycemia
- Pancreos (Innovita form code) — pancreatic beta-cell preservation

- Tri-Salts — metabolic acidosis correction (pH 5.43 + CO2 20)

Deal Breakers Triggered — All 7 + Liver

Deal Breaker #1: SNS SWITCHED

Evidence: PSNS -4.0 (worst in cohort), SNS +2.5 (highest in cohort), upper-left quadrant, dual "BE MORE ATTENTIVE."

- **Frequency:** SNS Balance 35@709

Deal Breaker #2: ALPHA/THETA IMBALANCE

Evidence: Theta 31% vs Alpha 8% — 4:1 ratio. Delta 56% approaching 70% threshold.

- **Frequency:** Alpha Theta
- **Supplements:** Cell Synergy **DOUBLE DOSE** (4 scoops), D-Ribose

Deal Breaker #3: PNS NEGATIVE ZONE

Evidence: PSNS -4.0, Index of discrepancy -3 (worst in cohort).

- **Frequency:** PNS Support

Deal Breaker #4: VAGUS DYSFUNCTION

Evidence: E/I 1.02 (flat), PSNS -4.0, PNS sharply decreased.

- **Frequencies:** Vagus Support + Vagus Balance + assess for Vagus Trauma
- **Supplement:** Innovita Vagus Nerve

Deal Breaker #5: LOW HEART (1%)

Evidence: Heart 1% — floor value. CRP 3.01, HDL 37, homocysteine 17.6, uncontrolled diabetes, CKD.

- **Frequency:** Heart Health
- **Supplements:** Serculate, CoQ10

Deal Breaker #6: LOW pH (5.43)

Evidence: pH 5.43 — below 6.2 mandatory Tri-Salts threshold. CO2 20 (metabolic acidosis).

- **Supplements:** Cell Synergy + Tri-Salts (MANDATORY)
- **Frequency:** Terrain

Deal Breaker #7: VCS FAILED (12/32)

Evidence: VCS 12/32 — critical fail.

- **Frequency:** Biotoxin
- **Supplements:** Pectasol-C (immediate); Apex (delayed)

Additional Deal Breaker: ELEVATED LIVER ENZYMES (AST 57)

Evidence: AST 57 (normal 0–40), ALT 44 (upper limit), LDH 258 (cellular injury). In context of HbA1c 9.6, this is almost certainly NASH (non-alcoholic steatohepatitis) — fatty liver inflammation. The liver is the metabolic command center; its compromise amplifies every other metabolic derangement.

- **Frequency:** Liver Inflame

- **Supplements:** Livergy, Rejuvenation H2

Frequency Protocols — Complete List (27 Total)

Phase 1: Deal Breakers (Sessions 1–4)

#	Frequency	Evidence
1	**SNS Balance 35@709**	PSNS -4.0, SNS +2.5, dual BE MORE ATTENTIVE
2	**Alpha Theta**	Theta 31% vs Alpha 8% (4:1 ratio)
3	**PNS Support**	PSNS -4.0, Index of discrepancy -3
4	**Vagus Support**	E/I 1.02, PSNS -4.0
5	**Vagus Balance**	Vagal dysfunction layered support
6	**Heart Health**	Heart 1%
7	**Terrain**	pH 5.43
8	**Biotoxin**	VCS 12/32
9	**Liver Inflame**	AST 57 — DEAL BREAKER

Phase 2: Organ/System Support (Sessions 3–8)

Frequency	Evidence
Sacral Plexus	Sacrum 1%
Cervical Support	Cervical 3%
Kidney Repair / Kidney Vitality	eGFR 50, Creatinine 1.64, Phosphorus 4.3 — begin Session 3
Pineal Support	Energy 11 — melatonin lever
Pit A Support + Pars Intermedia	VCS 12/32 → MSH restoration
Vein Vitality	ChMR 0.95, absent vascular compensation
Concussion	Upper-left quadrant + brainwave pattern — assess TBI history

Phase 3: Lab-Driven + Condition-Specific (Sessions 5–12+)

Frequency	Evidence
Mito Leak	CRP 3.01, LDH 258
Insulin Resist	HbA1c 9.6, Glucose 151
Pancreas T2D	HbA1c 9.6, Urine Glucose 3+
SIBO (nightly)	T2D — gut-metabolic axis

Small Intestine	HDL 37, gut-liver axis
Liver Tox	HDL 37, AST 57, hepatic lipid metabolism
Ferritin Lower	Ferritin 369
Leptin Resist	Energy 11, all organs red
Aldehyde Detox	Uric Acid + eGFR 50 impaired clearance

Gated / Delayed

Frequency	Gate	Status
Cyto Lower	MSH must be confirmed >8	VCS 12/32 = MSH almost certainly suppressed. HARD GATE.
Vagus Trauma	Assess for trauma history	Add if trauma history confirmed

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Supplementation Stack

Day 1 — Non-Negotiable

Supplement	Dose	Indication
Cell Synergy	**4 scoops** (DOUBLE — BE MORE ATTENTIVE protocol)	Dual BE MORE ATTENTIVE, Theta 31% > Alpha 8%, pH 5.43
Tri-Salts	Per label — mandatory	pH 5.43 (below 6.2 mandatory threshold)
CoQ10	Standard (target RBC >1.65)	Heart 1%, CRP 3.01
Serculate	Per label	Heart 1%, cardiovascular
Pectasol-C	Per label	VCS 12/32, eGFR 50, HDL 37 — triple duty
Vitamin D3	10,000 IU + K2	Vitamin D 37.3 (target 60–80)

> ⚠️ ***Vitamin D caution with CKD:*** *10,000 IU D3 increases phosphorus absorption. Phosphorus is already 4.3 (elevated) and eGFR 50 limits phosphorus clearance. Monitor phosphorus and calcium every 2 weeks at this dose.*

First 2 Weeks

Supplement	Indication
Livergy	AST 57 — liver deal breaker
Rejuvenation H2	T2D + mitochondrial + liver
Pancreos (Innovita)	HbA1c 9.6, Glucose 151, Urine Glucose 3+
Homocysteine Factor	Homocysteine 17.6

IP6 Gold	Ferritin 369 — ⚠️ **EMPTY STOMACH ONLY**
Innovita Vagus Nerve	E/I 1.02, PSNS -4.0
Integra Cell	CRP 3.01
D-Ribose	Theta/Alpha imbalance, Energy 11
Kidney Clear	eGFR 50, CKD Stage 3a
Ion Gut Health	HDL 37, gut-liver axis

Ongoing (Week 3+)

Supplement	Indication
Epi Pineal	Energy 11
Silent Night patch	Melatonin nightly
X39 patch	12 hr on/off — UB Rates
Glutathione patch	Detox (VCS 12/32, liver compromise)
Hypothala	MSH/pituitary (VCS 12/32)
Adipothin	Leptin resistance
Fatty 15 (C15)	Cell membrane integrity
L-Ornithine L-Aspartate	Uric acid / aldehyde detox

Gated / Delayed

- **Apex:** Only once VCS trending upward from 12/32 — do not start now
- **Vitamin D maintenance:** After 4–6 weeks at 10,000 IU, retest D. Drop to 4,000 IU once >60.

Five Levers Status

Lever	Status	Action
Melatonin	🔴 Depleted (Energy 11)	Epi Pineal + Silent Night + Pineal Support
Leptin	🔴 Severely resistant (HbA1c 9.6, all organs red)	Adipothin + Leptin Resist
MSH	🔴 Almost certainly suppressed (VCS 12/32) — **GATES CYTO LOWER**	Pit A + Pars Intermedia + Hypothala + sunlight
Vitamin D	🟡 Suboptimal (37.3) — ⚠️ phosphorus caution with CKD	10,000 IU D3 + K2; monitor phosphorus and calcium bi-weekly
UB Rates	⚪ Not activated	X39 12 hr on/off

Diabetic Kidney Disease — Clinical Detail

TH has **established diabetic kidney disease (DKD)** — the leading cause of end-stage renal disease. Evidence:

Marker	Value	Implication
eGFR	50	CKD Stage 3a — 45% kidney function lost
Creatinine	1.64	Reduced glomerular filtration
Phosphorus	4.3	Early phosphorus retention — watch for >4.5
CO2	20	Kidneys failing to regenerate bicarbonate
Urine Glucose 3+	Critical	Osmotic diuresis — additional nephrotoxic stress on damaged nephrons
BUN	29	Upper end — early azotemia trending

The T2D protocol IS the kidney-preservation protocol. Every point reduction in HbA1c reduces the rate of kidney function decline.

Protocol Sequencing for CKD

- Kidney Repair/Vitality must begin at **Session 3** — not later
- Acidosis correction (pH 5.43, CO2 20) is kidney-protective — Tri-Salts + Terrain are both disease-modifying
- Pectasol-C: biotoxin binding + kidney clearance + cholesterol — essential triple duty
- **Apex delayed** — redistribution of mobilized toxins through eGFR 50 kidneys risks acute injury
- Monitor for phosphorus >4.5 (dietary phosphorus restriction) and >5.0 (conventional phosphate binders)
- Monitor potassium — CKD can impair potassium excretion; reassess any potassium-containing supplements if >5.0

Co-Management Required

This patient needs conventional medical co-management for T2D (endocrinology) and CKD (nephrology monitoring). HbA1c 9.6 with eGFR 50 is beyond the scope of any single modality. BFM provides the bioenergetic and nutritional framework — conventional medical management must be intensified in parallel.

Key Clinical Concerns — Top 5

1. Uncontrolled T2D With Active Organ Damage

HbA1c 9.6% + Urine Glucose 3+ = blood glucose consistently >180 mg/dL. Normal insulin (7.7) with glucose 151 indicates the pancreas is approaching exhaustion. Every day at HbA1c 9.6 increases nephropathy progression, neuropathy, retinopathy, and cardiovascular event risk.

2. Catastrophic Autonomic Collapse (Dual BE MORE ATTENTIVE)

PSNS -4.0 (worst in cohort), SNS +2.5 (highest in cohort), Energy 11, Stress Response 7. The body cannot rest, repair, digest, or regulate inflammation. SNS activation drives hepatic glucose output — autonomic

correction is directly metabolically beneficial.

3. CKD Stage 3a — The Ticking Clock

eGFR 50 = 45% kidney function lost. Stage 3a is still reversible if addressed aggressively. Stage 4 (eGFR <30) begins dialysis planning. The combination of uncontrolled diabetes + metabolic acidosis + phosphorus retention + osmotic nephrotoxicity from glucosuria creates a vicious cycle.

4. Biotoxin Burden + Suppressed MSH (VCS 12/32)

With MSH almost certainly suppressed, the body cannot self-regulate inflammation. This compounds diabetes inflammation, kidney inflammation, liver injury (AST 57), and cardiovascular inflammation (CRP 3.01). Environmental biotoxin source assessment is warranted (home/workplace mold, ERMI test).

5. Hepatic Injury (NASH Pattern — AST 57)

The liver is the metabolic command center. NASH in the context of HbA1c 9.6 dysregulates glucose output (hepatic gluconeogenesis), lipid metabolism (HDL 37), homocysteine clearance (17.6 elevated), and detoxification. Liver Inflame frequency + Livergy must be Phase 1 priorities.

Monitoring Recommendations

Session Frequency

Minimum 2× per week (ideally 3×/week first 4–6 weeks per dual BE MORE ATTENTIVE). Reassess at 6 weeks.

Priority Retest Schedule

Timeframe	Test	Threshold for Concern
2 weeks	DePulse + HRV/Ortho/Valsalva	No PSNS movement = protocol not penetrating
2 weeks	Urinalysis pH	Still <6.0 after Tri-Salts = reassess acidosis driver
4 weeks	LabCorp BMP (glucose, creatinine, eGFR, CO2, phosphorus, K)	eGFR <45 = URGENT escalation; Phosphorus >4.5 = diet intervention
4 weeks	HbA1c	Target <8.5 at 4 weeks
4 weeks	VCS	Should move from 12/32 toward 16+/32
6 weeks	Full LabCorp (CMP, lipids, CRP, homocysteine, ferritin, liver enzymes, LDH)	AST trending <45 · CRP <2.5 · Ferritin <336 · Homocysteine <15
6 weeks	Vitamin D retest	Should be >50
8 weeks	MSH	**Gate for Cyto Lower activation**
8 weeks	UACR (spot urine albumin-to-creatinine)	Quantify proteinuria for DKD staging
12 weeks	Full comprehensive reassessment	All systems, all labs, all Five Levers

Red Lines — Immediate Escalation

Trigger	Action
eGFR drops below 40	Nephrology referral
Glucose consistently >200 or DKA symptoms	Emergency medical evaluation
AST rises above 100	Hepatology referral
New chest pain, dyspnea, syncope	Cardiology / ED
Potassium >5.5	Immediate medical evaluation (cardiac risk)

Cross-Patient Notes & Shared Protocols {#cross-patient-notes}

Cyto Lower Gate — All 4 Critical Patients

All four severe patients (DH, GD, GG, TH) present with the electron spin inversion / Krebs reversal pattern (all organs in single-digit range) and require Cyto Lower. **None can run it yet.**

Patient	VCS	MSH Status	Gate Status
DH	2/32	Almost certainly <8	🔴 GATED — begin Pit A + Pars Intermedia + Hypothala
GD	3/32	Almost certainly critically low	🔴 GATED
GG	8/32	Likely suppressed	🔴 GATED
TH	12/32	Almost certainly suppressed	🔴 GATED
JW	25/32 (PASSING)	Likely borderline but not suppressed	🟢 Cyto Lower may not be needed

Protocol for all four gated patients:

1. Begin Pit A Support frequency + Pars Intermedia frequency now
2. Add Hypothala supplement now
3. Morning sunlight protocol (10–15 min within first hour of waking)
4. Obtain MSH blood level
5. Recheck MSH at 4–6 weeks after starting protocol
6. Cyto Lower can ONLY be added once MSH is lab-confirmed >8

Cyto Lower duration when approved: **1 hour 45 minutes.**

IP6 Gold — Empty Stomach Protocol (DH, TH)

> ⚠️ ***CRITICAL INSTRUCTION:*** *IP6 Gold must be taken on an* ***EMPTY STOMACH*** *— at least 1 hour before food or 2 hours after eating. If taken with food, efficacy is significantly reduced. No exceptions.*

- DH: Ferritin 389 → IP6 Gold
- TH: Ferritin 369 → IP6 Gold

Vitamin D Protocol — All Patients With Suboptimal Levels

Loading dose: 10,000 IU D3 daily × 4–6 weeks **K2 (MK-7) co-administration:** 200 mcg per 1,000 IU of D3 (mandatory — prevents calcium misplacement) **Retest:** 25-OH Vitamin D at 6 weeks. If >60: reduce to 4,000 IU maintenance + K2 800 mcg.

Patient	Current Vitamin D	Action
DH	41.9	10,000 IU + K2 loading
GD	Unknown	Obtain labs first
GG	Unknown	Obtain labs first
JW	Unknown	Obtain labs first
TH	37.3	10,000 IU + K2 — ⚠️ monitor phosphorus bi-weekly (eGFR 50)

Labs Still Needed — Summary

Patient	Most Urgent Labs
DH	MSH · RBC CoQ10 · Spike Ab trend at 6 weeks
GD	Everything — CMP · MSH · 25-OH Vit D · Magnesium RBC · CBC · CRP · Cortisol · Thyroid
GG	CMP with GFR (MOST URGENT — proteinuria 3.0 + Kidneys 1%) · MSH · Albumin/Total Protein
JW	25-OH Vitamin D · CMP (Sp. Gravity 1.005) · CRP · TSH · Leptin
TH	UACR · MSH · RBC CoQ10 · Potassium monitoring · Phosphorus bi-weekly

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Session Frequency Recommendations

Patient	Recommended Frequency	Reason
DH	Weekly (minimum)	Urgency 5/5, close monitoring required
GD	Weekly (minimum); bi-weekly if tolerated	Urgency 5/5, arrhythmias, physiological resources 4/600
GG	Weekly	Urgency 5/5, paradoxical Vagus Trauma retest required at Week 4

JW	Weekly to bi-weekly	Urgency 2.5/5, monitoring for trajectory changes
TH	**Minimum 2×/week**	**Dual BE MORE ATTENTIVE mandate**

Protocol Reference — Key Supplements Across Cohort

Supplement	DH	GD	GG	JW	TH	Notes
Cell Synergy	2–3 scoops	2–3 scoops	1–2 scoops	1 scoop	**4 scoops**	BE MORE ATTENTIVE = double
Tri-Salts	✅ Mandatory (pH 5.08)	✅ Mandatory (pH 4.89)	Conditional (pH 5.79)	❌ Not needed (pH 6.43)	✅ Mandatory (pH 5.43)	Mandatory when pH <6.2
Pectasol-C	✅	✅	✅	Hold unless VCS drops	✅	All critical VCS failures
Augmented NAC	✅ 3/day × 5–6 mo	❌	❌	❌	❌	Spike protein specific
IP6 Gold	✅ Empty stomach	❌	❌	❌	✅ Empty stomach	Ferritin elevated
CoQ10	✅	✅	✅	❌	✅	Target RBC >1.65
Serculate	✅	✅	✅	❌	✅	Heart 1–2%
Livergy	❌	❌	❌	❌	✅	AST 57 deal breaker
Pancreos	❌	❌	❌	❌	✅	T2D only
Apex	Delayed	Delayed	Delayed	Monitor	Delayed	All — after VCS improving
X39 patch	✅	✅	✅	✅	✅	All patients — 12hr on/off

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